

PROLIDIN SYRUP

MAL19910177AZ

DESCRIPTION:

SYRUP

Colour : Red
Flavour : Cherry

CONTENT:

Each 5 ml Contains:

Tripolidine HCl..... 1.25 mg

Pseudoephedrine HCl 30 mg

Preservatives: Sodium Benzoate 0.1% w/v & Methyl Paraben 0.1% w/v & Propyl Paraben 0.01% w/v.

PHARMACODYNAMICS:

Tripolidine HCl

It is an antihistamine which acts by competing with histamine for H₁-receptor, but do not reverse responses mediated by histamine alone. The antimuscarinic action provides a drying effect on the nasal mucosa.

Pseudoephedrine HCl

It is a sympathomimetic amine with direct and indirect effects on adrenoreceptors. It constricts the peripheral vessels, thus raising the blood pressure. It relaxes the bronchioles and decreases the tone and peristaltic movements of the intestine but contracts the uterus.

PHARMACOKINETICS:

Tripolidine HCl is absorbed after oral administration and distributes throughout the body. It is metabolised in the liver and excreted in the urine.

Pseudoephedrine HCl is rapidly and completely absorbed after oral administration. After an oral dose of 180 mg, peak plasma conc. of 500 to 900 ng/ml are attained in 1 to 3 hours. Plasma half-life after an oral dose is 5 to 8 hours which may be increased in subjects with alkaline urine and decreased in subjects with acid urine. The volume of distribution is 2 to 3 litres/kg body-weight. It is metabolised by N-demethylation to a small extent. About 70% of a dose is excreted in the urine in 36 hours with less than 1% as norpseudoephedrine and the remainder as unchanged drug.

INDICATIONS:

For the temporary relief of nasal and sinus congestion, sneezing and rhinorrhoea associated with the common cold and allergic rhinitis.

RECOMMENDED DOSAGE:

Adults and children over 12 years : 2 teaspoonfuls (10 ml).

Children 7 - 12 years : 1 teaspoonful (5 ml).

Children 2 - 6 years : ½ teaspoonful (2.5 ml).

All to be taken 4 to 6 hourly.

WARNING (When Used For Treatment of Cough and Cold):

1. Not to be used in children less than 2 years of age.
2. To be used with caution and doctor's / pharmacist's advice in children 2 to 6 years of age.

ROUTE OF ADMINISTRATION:

FOR ORAL USE ONLY

CONTRAINDICATIONS:

The preparation should not be used for patients treated with MAO inhibitor. It is contraindicated in coronary thrombosis and thyrotoxicosis.

WARNING/PRECAUTIONS:

Care should be taken when given to patients with cardiac disease, hypertension, or diabetes mellitus. It may cause drowsiness and if affected, one should not drive or operate machinery. Patients intolerant of other antihistamines or other sympathomimetics. Use of antihistamines is not recommended in new-born or premature infants. This age group may be at higher risk than other age groups because of an increase susceptibility to antimuscarinic effects, such as CNS excitation, and an increase tendency toward convulsion. Prolonged use of antihistamines may decrease or inhibit salivary flow, thus contributing to the development of caries, periodontal disease, oral candidiasis and discomfort.

DRUG INTERACTIONS:

Pseudoephedrine may cause hypertensive crisis when given together with tricyclic antidepressants. It may antagonise the hypotensive effects of adrenergic neurone blocker antihypertensives. There is a possibility of severe hypertensive effect when concomitantly used with beta-blockers.

Alcohol, MAOI and tricyclic antidepressants, and anxiolytics and hypnotics may enhance the sedative effects of Tripolidine. The later two may also increase the antimuscarinic effects. Concurrent use with other antimuscarinics may exaggerate the side effects. There is also a theoretical antagonism effect between Tripolidine and betahistine.

PREGNANCY AND LACTATION:

Care should be taken when given to pregnant women. Use is not recommended in nursing mothers because of the risk of antihistamine causing excitement or irritability in infants, and the higher than usual risk for infants from sympathomimetic amines. Antihistamines may inhibit lactation because of their antimuscarinic action.

SIDE EFFECTS/ADVERSE REACTIONS:

Drowsiness, dizziness, muscular weakness, gastrointestinal disturbances. It may raise the blood pressure. In patients with prostatic hypertrophy it may give rise to urinary retention.

Sore throat, fever, unusual bleeding or bruising, unusual tiredness or weakness, tightness in chest, fast or irregular heartbeat, mood or mental changes, clumsiness or unsteadiness, dryness of mouth, nose, throat, severe or flushing or redness of face, shortness of breath, hallucinations, seizures, trouble in sleeping, headache.

SYMPTOMS AND TREATMENT OF OVERDOSE:

Symptoms: Drowsiness, lethargy, dizziness, ataxia, weakness, hypotonicity, respiratory depression, dryness of the skin and mucous membranes, tachycardia, hypertension, hyperpyrexia, hyperactivity, irritability, convulsions, difficulty in micturition. In severe overdosage, apnoea, circulatory collapse, cardiac arrest and death may occur.

Treatment: Necessary measures should be taken to maintain and support respiration and control convulsions. Gastric lavage should be performed up to 3 hours after ingestion, if indicated. Catheterisation of the bladder may be necessary. If desired, the elimination of Pseudoephedrine can be accelerated by acid diuresis or by dialysis.

PACKING/PACK SIZE(S):

Plastic bottle of 60 ml.

JAUHI UBAT DARIPADA KANAK-KANAK

KEEP OUT OF REACH OF CHILDREN

SHAKE WELL BEFORE USE

MANUFACTURER/PRODUCT REGISTRATION HOLDER:

DYNAPHARM (M) SDN. BHD. 198001011897 (65683-V)

2497, Mk. 1, Lorong Perusahaan Baru 5,

Kawasan Perusahaan Perai 3,

13600 Perai, Pulau Pinang, MALAYSIA

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